The Hidden Cost of Trying Harder

Internal control appraisals under external adversity: resilience or masked strain?

Core question: When a difficulty is caused from the outside, is coping by reappraising it and attributing it to oneself ("I'm not trying hard enough") real resilience, or a suppression whose costs self-reports don't show?

Background

Cognitive coping is usually seen as a resource. Strong reappraisal ability, the skill of reinterpreting a situation so that it feels different, buffers the effect of stress on depression (Troy et al., 2010). Attributing difficulty to internal, controllable causes such as effort is also considered adaptive, because it implies the outcome can be changed by trying harder (Weiner, 1985).

But some difficulties are caused from the outside. At the root, these can only be resolved by external action: problem-focused coping, which changes the situation, rather than emotion-focused coping, which changes one's response to it (Lazarus & Folkman, 1984). Strong cognitive skills may still let people cope, but the cause stays in place, and I suspect this may ultimately be worse than solving the problem at its root. Consistent with this, higher reappraisal ability went with fewer depressive symptoms under uncontrollable stress (such as a partner's illness), but *more* symptoms under controllable stress (such as problems at work) (Troy, Shallcross & Mauss, 2013).

Self-reports may miss the cost. The evidence that these skills are a resource rests mostly on self-reported distress. I want to find out whether coping can lower reported distress while the body keeps paying; if so, self-report alone would mistake strain for resilience.

Study design

Real situations are ambiguous. They rarely sort cleanly into personal responsibility or structural unfairness; mostly both apply at once, and both readings have benefits. In everyday life, the appraisal and the situation therefore vary together, and their effects can't be separated. The study holds the situation constant and induces the appraisal experimentally: both groups face the same, equally unfair difficulty and differ only in how it is framed.

	"It's on me" group (internal)	"It's unfair" group (external)
Unfairness	Unknown to participants	Known to participants
Induced appraisal	"I am not trying hard enough"	"The situation is just unfair"

Hypotheses

What prior work suggests

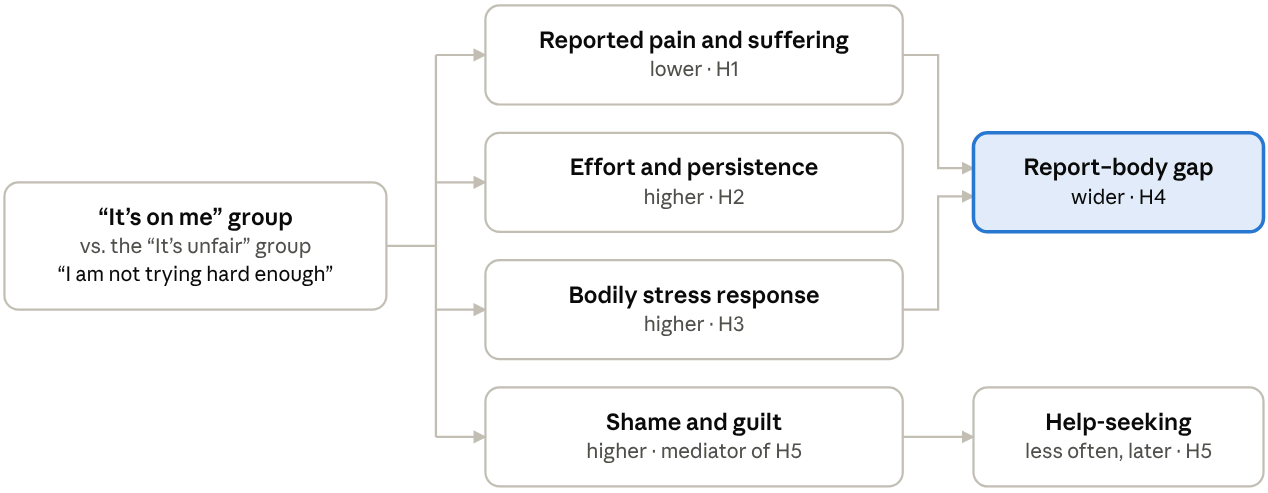
- **H1: Suffering.** I expect the "It's unfair" group to report more pain and suffering caused by the difficulty. Identical harms hurt more when seen as intentional (Gray & Wegner, 2008), and perceived injustice predicts worse pain outcomes (Sullivan et al., 2008).
- **H2: Effort.** I expect the "It's on me" group to try harder, investing more effort and persisting longer, since attributing failure to a lack of effort sustains persistence (Dweck, 1975; Weiner, 1985).

What I want to test

- **H3: Bodily cost.** Does the "It's on me" group show a stronger physiological stress response, for example cardiovascular, hormonal or inflammatory strain? This seems plausible, since inducing self-blame has been shown to raise shame and proinflammatory activity (Dickerson et al., 2004).
- **H4: Widening gap.** Do reported suffering and bodily cost drift further apart in the "It's on me" group? This is where resilience and masked strain can be told apart. If both are low, the coping looks genuinely resilient; if only the report is low, it may be a less healthy suppression masked by a coping style. Either result would be informative. Self-report and physiology are known to diverge (Weinberger et al., 1979; Gross & Levenson, 1993).
- **H5: Help-seeking.** Does the "It's on me" group seek help less or later, and do shame and guilt explain this? One possible reason: if the problem feels self-caused, asking for help may feel like admitting failure. Receiving help can threaten self-esteem (Nadler & Fisher, 1986), and seeking it carries self-stigma (Vogel et al., 2006).

Does the “It’s on me” group report less suffering but pay more in the body?

Expected direction of each outcome in the “It’s on me” group, relative to the “It’s unfair” group



predicted model · five hypotheses

Relevance

If these effects hold, one mechanism could help explain several known phenomena. In each, an internal appraisal of an external or unrecognised difficulty seems to keep people functioning, while the costs may stay hidden and surface later.

- **Late ADHD and autism diagnosis.** Many people spend years thinking "I just need to try harder, this is normal", which keeps them going. Diagnosis shifts this to "I have a condition, it's not my fault, and I deserve help": the appraisal switches from internal to external. Many feel relief, but also anger and grief that was not there before (Young et al., 2008; Leedham et al., 2020), a reaction one study called a "not guilty verdict" (Punshon et al., 2009). For some, functioning suddenly stops: a decompensation, meaning a breakdown of coping that had held until then, resembling autistic burnout (Raymaker et al., 2020).
- **Disadvantaged groups.** Under structural disadvantage, competence and self-control predict good psychosocial outcomes, but also more physiological wear and tear (allostatic load) and faster biological aging of cells (epigenetic aging). The authors call this resilience "skin deep": visible on the surface, but not in the body (Brody et al., 2013; Miller et al., 2015).

- **Adverse childhood.** People who grew up in adverse circumstances often carry self-blame, guilt and shame. In that environment self-blame was adaptive: "it was my fault" preserves a sense of control, which is less frightening than helplessness (Janoff-Bulman, 1979). Healing often comes from realising the situation was unfair; only then can grief and anger surface (Herman, 1992).

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